Northbridge General Hospital (fictional)

100 Example Avenue, Northbridge - fictional address, telephone 000 0000

Medical Oncology Consultation

Department of Medical Oncology

PATIENT **Thomas Whitfield**	UHID **MG25006**	AGE / SEX **49 years / Male**
REPORT DATE **12 Mar 2026**	REFERENCE NO. **CON-77503003**	DEPARTMENT **Department of Medical Oncology**

Reason for Visit

Discussion of adjuvant treatment after laparoscopic sigmoid colectomy on 10 Feb 2026 for sigmoid colon adenocarcinoma. Mr. Whitfield recovered well and has returned to part-time work.

Histopathology

Moderately differentiated adenocarcinoma, **pT3 pN1 (2 of 18 nodes) M0, stage III**. Resection margins clear (R0). Lymphovascular invasion present. Microsatellite stable.

Pertinent negatives: No history of inflammatory bowel disease. No family history of colorectal cancer. No known heart disease.

Baseline

- ECOG performance status 0
- Weight 82 kg, body surface area 1.98 m2
- Creatinine 0.9 mg/dL, normal liver tests

Allergies

No known drug allergies.

Tumour Marker

Test	Result	Reference
Carcinoembryonic antigen (CEA)	6.8 ng/mL	< 5.0 ng/mL

Treatment Plan

Adjuvant **CAPOX** (capecitabine plus oxaliplatin), eight three-weekly cycles starting on 30 Mar 2026. Doses calculated for body surface area 1.98 m2.

Drug	Dose per m2	Dose given	Schedule
Capecitabine	1000 mg/m2	2000 mg twice daily (BD)	Days 1 to 14, oral
Oxaliplatin	130 mg/m2	255 mg	Day 1, intravenous infusion

Plan

- Chemotherapy education and consent completed today.
- Blood count and liver and renal function before every cycle.
- Counsel on hand-foot syndrome, diarrhoea and cold-induced neuropathy.
- CEA every 3 months during and after treatment.

Electronically signed: **Dr. Rebecca Tan, Consultant Medical Oncologist** on 12 Mar 2026

*** End of report ***

SYNTHETIC DATA — FOR DEMONSTRATION ONLY